

CFS/ME SERVICES FOR SOUTH STAFFORDSHIRE – REFERRAL FORM

Please note that the service DOES NOT have medical input and therefore all diagnoses MUST be completed BEFORE referral to the service (see form for diagnostic criteria).

Please complete all sections of the form (Sections 1-4). Incomplete referral forms will be returned.

1. PATIENT DETAILS

Surname:	Surname	Title:	Title
Forename(s):	Given Name	Date of Birth:	Date of Birth
Address:	Home Full Address (single line)		
Telephone Number:	Patient Home Telephone Patient Mobile Telephone	NHS No:	NHS Number
Ethnicity:	Ethnic Origin		

2. REFERRER DETAILS

Name:	Free Text Prompt
Address:	Organisation Full Address (single line)
Telephone Number:	Organisation Telephone Number
Email Address:	Organisation E-mail Address

3. DIAGNOSIS

Please note that the service DOES NOT have medical input and therefore all diagnoses MUST be completed BEFORE referral to the service (Criteria from NICE guidance are given below to aid with diagnosis, however please note that diagnosis is the responsibility of the referrer).

Criteria for diagnosis – symptoms of three months duration or more (please tick)

3.1 The fatigue should have all the following features:

• Persistent or recurrent	☐
• Of new or specific onset, but at least 3 months duration	☐
• Unexplained by other conditions (e.g. inflammation, severe mental illness, chronic disease)	☐
• Characterised by post-exertion malaise and/or fatigue (typically delayed, for example by at least 24 hours, with slow recovery over several days)	☐
• Causing a substantial reduction in activity level (affecting occupational, educational, social or personal activities)	☐

3.2 In addition to fatigue, are one or more of the following symptoms present?

• Difficulty with sleeping (insomnia, hypersomnia, unrefreshing sleep, disturbed sleep-wake cycle)	☐
• Muscle and/or joint pain that is multi-site and without evidence of inflammation	☐
• Headaches	☐
• Painful lymph nodes without pathological enlargement	☐
• Sore throat	☐
• Cognitive dysfunction (difficulty thinking, inability to concentrate, impairment of	☐

short-term memory difficulty with word-finding, planning/organising thoughts and information processing)	
• Physical or mental exertion makes symptoms worse	☐
• General malaise or 'flu-like' symptoms	☐
• Dizziness and/or nausea	☐
• Palpitations in the absence of identified cardiac pathology	☐

3.3 Have you investigated the following 'Red flag' features? These should not be attributed to CFS/ME without consideration of alternative diagnoses or comorbidities.

• Localising/focal neurological signs	☐
• Signs and symptoms of inflammatory arthritis or connective tissue disease	☐
• Signs and symptoms of cardio respiratory disease	☐
• Significant weight loss	☐
• Sleep apnoea	☐
• Clinical significant lymphadenopathy	☐

Reconsider the diagnosis if the person does not have any of the following symptoms:

• Post-exertional fatigue or malaise	☐
• Sleep disturbance	☐
• Cognitive difficulties	☐
• Chronic Pain	☐

3.4 Investigation Protocol

(referrals will not be accepted without the following completed)

Blood tests to be carried out prior to referral = please tick to indicate that these have been completed and normal results obtained

Urinalysis for protein blood and glucose	☐	Erythrocyte sedimentation rate or plasma viscosity	☐	Creatinine kinase	☐
Full blood count	☐	C-reactive protein	☐	Thyroid function	☐
Urea and electrolytes	☐	Random blood glucose	☐	Serum calcium	☐
Liver function	☐	Screening tests for gluten sensitivity	☐	Assessment of serum ferritin levels (children and young people only)	☐

3.5 Confidence in Diagnosis

How confident are you of the diagnosis of Chronic Fatigue Syndrome/ME?

Not very ☐ Reasonably ☐ Very ☐

3.6 Patient Agreement

Does the patient agree with the diagnosis?

Yes ☐ No ☐

3.7 Symptom Duration

Months since onset of symptoms (*patient defined*) months

3.8 Severity

Please indicate the severity of CFS/ME for this patient (*see criteria below*)

Mild ☐ Moderate ☐ Severe ☐

- **Mild:** Mobile, self-caring, light domestic tasks with difficulty. Still working or in education, but to do this they have probably stopped all leisure activities
- **Moderate:** reduced mobility, restricted in all activities of daily living, peaks and troughs of activity and symptoms. Not working or in education. Need rest periods. Poor sleep.
- **Severe:** unable to do any activity for themselves or can carry out minimal daily tasks only. Severe cognitive difficulties. Depend on wheelchair for mobility. Housebound or bed-bound most of the time. Light and noise intolerant.

4. CLINICAL INFORMATION

4.1 Clinical Summary *(Please provide information on current and past medical history, symptoms, examinations and findings, previous or concurrent psychiatric illness, alcohol or unprescribed drugs/ medications, current prescribed medication, social situation, other services involved, any known risks/vulnerability and employment. If this is a re-referral, please state the reasons. Please enclose a referral letter if required).*

4.2 Current Medications

See below

4.3 Reason for referral

Signed		Date	Short date letter merged
Print name		Title	
Address	Organisation Full Address (single line)		
Email	Organisation E-mail Address		

Problems
Medication
Allergies

Please send this referral to:-

**Clinical Lead, CFS/ME Services for South Staffordshire,
Physical Health Psychology Department, Therapy Services, Level 2, Cannock Chase
Hospital, Brunswick Road, Cannock, Staffs, WS12 5XY**

OR email it to physicalhealthpsychologyservice@mpft.nhs.uk